From "Everyone Will Pitch In" to a Name Beside Every Gap

An ordered plan for the days before your husband's care changes.

The goal for these few days: one page that shows what you've noticed, which hours are covered and which aren't, and who is following up on each, ready for the care team to look at.

You don't have to do every line yourself. Put a name and a day beside each action, and tick it off when it's done.

ACTION	WHO	BY WHEN	DONE
1. Collect the instructions the care team has already given, in writing where you can, and note the date on each.			
2. Write out an ordinary day in blocks, early morning through overnight. Put a name in a block only where someone has confirmed days and times.			
3. Mark the blocks with nobody in them, overnight included.			
4. For each covered block, note who steps in if that person can't come. Leave it empty where there is no one.			
5. Write down which hours fall to you, and which of them you can keep up.			
6. Write down what you've seen about falls or how he moves around: what happened, where and when.			
7. Note any difficulty with his medications as something for the pharmacist or the prescriber to review. Change nothing.			
8. Note anything about the place he's going that you've seen or been told about.			
9. Turn each open safety worry into a question, and take it to the professional it belongs to: the discharge planner, the nurse, the pharmacist or the therapist.			
10. Beside every concern and every open block, write who is following up and by when.			

You're done when

The concerns you've noticed and the hours nobody has covered are on paper, each with a name beside it, and every open safety question has gone to the professional it belongs to. A question you've passed on is still open until someone answers it. Reviewing those questions, and any decision about the arrangement, comes after this list and belongs to the people who assess your husband.

If the days run short

- A blank is a finished line. Write “nobody yet” and leave it showing.
- Take the page as it is. An unfinished page with honest blanks on it is still worth bringing.
- If you're feeling hurried, the Family Caregiver Alliance says it is reasonable to ask the discharge planning team to slow things down so nothing is overlooked.
- If he falls, or seems unsteady, don't save it for this page. The CDC's advice to family caregivers is to tell a healthcare provider right away.

What this plan doesn't do

This plan covers the family's part and stops there. Finishing it doesn't certify that the arrangement is safe or authorize any change to his care, and it doesn't replace the professional review it leads up to.

Ready for the next step?

You've spent these days getting the concerns and the open hours onto paper, each with a name beside it. The care interview goes through the rest with you, area by area: what your husband can do, who is caring for him and what the care team has said. It turns your answers into a Family Care Profile you can bring to any care conversation.

Create Your Free Family Care Profile ›

WHERE THIS COMES FROM

The lines about asking for more time and about telling a provider after a fall follow the Family Caregiver Alliance's discharge guide and the CDC's brochure for family caregivers. The actions themselves come from the Family Care Roadmap's working list for this step.

1. Family Caregiver Alliance, Hospital Discharge Planning: A Guide for Families and Caregivers
2. Centers for Disease Control and Prevention, Family Caregivers: Protect Your Loved Ones from Falling (STEADI brochure, 2018)

From the Family Care Roadmap · Richard Ladson, Certified Medicaid Planner. Draft for review, generated 2026-10-04.